

In philosophy, the *Lottery Paradox* asks: **If you know one ticket will win, but each ticket is unlikely to win, is it rational to believe each ticket will lose?**

It's a cautionary tale about how we misjudge low-probability, high-impact events.

In healthcare, self-funded employers fall into the same trap every day, especially with organ transplants, gene therapies, and CAR-T treatments. The cost of this thinking? Seven-figure claims that blow past stop-loss.

“It’s Unlikely to Happen to Us...”

Employers aren't wrong. Each catastrophic event is rare. But here's the paradox: **Individually unlikely ≠ Collectively avoidable.**

With the pace of medical innovation, the probability that *something* high-cost will hit your plan is not “if,” it's “when.”

You're not betting on a single ticket. You're holding the *entire* raffle.

For Self-Funded Employers and Their Advisors: This Is Not Theoretical

If you're a benefits consultant or broker, you've likely heard “These treatments are edge cases, so we don't need to prioritize them.”

The data says otherwise:

- 69,000+ U.S. transplants each year
- CAR-T therapy use growing rapidly, especially in younger cancer patients
- Gene therapies accelerating, with multiple \$2–3M FDA-approved treatments already in play

For groups with 5,000+ members, these cases are no longer hypothetical. They're actuarial certainties.

When Rare Hits, It Hits Hard

High-cost medical events:

- Routinely blow through stop-loss budgets
- Trigger out-of-network costs (even at “in-network” hospitals)
- Are often managed via carve-out networks with poor transparency
- Can vary by \$200K–\$1M+ depending on routing and contract type

At HealthRate, we see:

- The *same* hospital billing wildly different rates for the *same* transplant based on referral source
- Gene therapies marked up by hundreds of thousands (sometimes millions)
- Specialty networks charging 20–40% more than direct contracts for identical cases

How Advisors Use HealthRate to Win on Risk Management

We help you intervene before the bill arrives by:

- Flagging internal provider rate variance across networks
- Modeling stop-loss exposure *before* the claim hits
- Comparing clinical outcomes and reimbursement rates
- Identify opportunities for high-quality, lower cost services

This Is an Advisory Moment

For brokers and consultants, managing million-dollar claim risk isn’t optional—it’s where strategic value is proven.

With HealthRate, you shift the client conversation from *“That’ll probably never happen”* to *“Here’s exactly how we’re protecting your plan from it.”*

Bottom Line: The Paradox Holds

Each event may be rare. But *some* event will happen.

The only irrational move? Believing otherwise.

Ready to protect your plan from the next million-dollar surprise?

If you’re a benefits consultant or self-funded employer, contact us for a free exposure analysis, or preview our analytics tools in action.